

Emergency Medicine Billing & Coding

Level	Complexity	Descriptor
1	None	ED visit that may not require the presence of a physician or other qualified health care professional.
2	Straightforward	ED visit requiring a medically appropriate history and/or examination and straightforward medical decision making.
3	Low	ED visit requiring a medically appropriate history and/or examination and low medical decision making.
4	Moderate	ED visit requiring a medically appropriate history and/or examination and moderate medical decision making.
5	High	ED visit requiring a medically appropriate history and/or examination and high medical decision making.

Medical Decision-Making Criteria

**Level is determined by meeting criteria in 2 of the 3 MDM elements.

**The highest two of the three elements determine the overall complexity.

- 1: Number and Complexity of **Problems**
- 2: Complexity of **Data** Reviewed
- 3: **Risk** of Complications and/or Morbidity or Mortality

Level 4

Requires Level 4 MDM in 2 of 3 elements.

Complexity of Problems

- Chronic illness with exacerbation
- Acute illness with systemic symptoms (e.g., fever, nausea, vomiting)

Data: Must meet at least one of:

Category 1 - Tests/Documents/Historian (3 data points needed):

- Each unique ordered test = 1 point (e.g., CBC, CMP, troponin = 3 points)
- Reviewing prior external records = 1 point
- Using an independent historian (e.g., EMS, family) = 1 point

Category 2 - Independent Interpretation of Tests:

- Independent Interpretation of ECG, X-ray, POCUS, or cardiac monitor
- Note: EKG Currently does not count for us because we bill for this separately

Category 3 - Discussion of Care with External Provider:

Risk

- **Prescription drug management**
- Diagnosis or treatment significantly **limited by social determinants of health**
 - Examples: homelessness, unemployment, lack of insurance, substance abuse
- **Consideration to initiate testing, treatment, or hospitalization** – Even if you didn't do it, you can bill for it if you document thought process
ex: Discussed hospitalization however patient would like to go home and has close PCP Follow up

Level 5

Requires Level 5 MDM in 2 of 3 elements.

Complexity of Problems

- **Chronic illness with severe exacerbation**
- **Acute or chronic illness or injury that poses a threat to life or bodily function**

Data: Must meet at least 2 of the following:

Category 1 - Tests/Documents/Historian (3 data points needed):

- Each unique ordered test = 1 point
- Reviewing prior external records = 1 point
- Using an independent historian = 1 point

Category 2 - Independent Interpretation of Tests:

- Independent Interpretation of X-ray, POCUS, or cardiac monitor

Category 3 - Discussion of Care with External Provider:

Risk

- **Drug therapy requiring intensive monitoring** for toxicity
(Vasopressors, Amiodarone, Narcotics)
- **Decision regarding hospitalization or escalation of care**
- **Decision not to resuscitate or hospice/ comfort care**
- Diagnosis or treatment significantly **limited by social determinants of health**
- Decision regarding Surgery

Examples: Many Ways to Document Correctly

A few quick tips before the examples:

1. I would suggest having lab/radiology results auto-flow into your chart, and putting a statement above them that reads "I have ordered and interpreted the following lab testing:" This prevents you from having to list off every test in your MDM.
2. You cannot auto-flow EKG, CXR, POCUS, etc. - you must write your own independent interpretation to use these as a data point.

Turning a Level 3 into a Level 5

Level 3 MDM:

Patient presents with atypical chest pain x 2 days, worse with movement, seemingly musculoskeletal. Patient EKG is nonischemic, NSR, with no acute ST changes, Troponin <6, My interpretation of CXR demonstrates no infiltrate or consolidation, and he is low risk per heart score. CBC and CMP were also reviewed and unchanged. I believe this pain is likely musculoskeletal however patient will follow up with Low risk chest pain clinic. We discussed return precautions for any worsening of symptoms.

This is not a bad MDM from a legal standpoint, but it lacks a lot for billing:

3. Only possible medical problem listed is costochondritis. That's not life threatening (level 5) and not a systemic illness or worsening of a chronic condition (level 4) — so it's level 3 for Problem.
4. It DOES achieve level 5 for Data (ordered and reviewed 3 labs + CXR interpretation).
5. It does not consider hospitalization, list drugs requiring monitoring (level 5), or list medications prescribed (level 4) — so it's level 3 for Risk.
6. **This achieves level 5 for Data, but only level 3 for Problem and Risk. Since you need 2 of 3, this bills as a level 3.**

Level 5 MDM:

Same patient. Add the highlighted phrases and it's a level 5:

Patient presents with atypical chest pain x 2 days, worse with movement. **I considered and ruled out life threatening causes such as Acute Coronary Syndrome**, though this is seemingly musculoskeletal likely consistent with costochondritis. Patient EKG is nonischemic, NSR, with no acute ST changes, Troponin <6, CXR demonstrates no infiltrate or consolidation, and he is low risk per heart score, **making ACS an unlikely cause of this patient's symptoms**. CBC and CMP were also reviewed and unchanged. I believe this pain is likely musculoskeletal **however we discussed admission for further testing**. The patient would like to go home and follow up with the Low-risk



chest pain clinic which is reasonable as he is low risk per heart score. We discussed return precautions for any worsening of symptoms.

7. By adding a potentially dangerous diagnosis that you considered (ACS) you make this a Level 5 for Problem.
8. By adding the consideration of admission, you make this a Level 5 for Risk.
9. By having 3 labs ordered/reviewed and 1 independent interpretation of CXR you get Level 5 for Data (2 of 3 data points).
10. **This MDM is now level 5 in all 3 categories. You only need 2 of 3 to bill level 5.**

Level 5 MDM (Same Patient - Bullet Point Style):

- Patient presents with condition possibly posing threat to life or bodily function: Chest pain r/o ACS, Pulmonary embolism, possible costochondritis
- Ordered and reviewed CBC, CMP, Troponin <6
- EKG: NSR, Rate 86, no acute ischemic change
- Independent interpretation of Xray: No infiltrate or consolidation
- Decision regarding hospitalization: Patient low risk per heart score, considered admission however EKG and troponin nonischemic and patient would like to go home. No Pleuritic pain, hypoxia, SOB, or risk factors to suggest Pulmonary Embolism. Patient will follow up with low-risk chest pain clinic in 2-3 days, we have discussed return to ER for any resumption of symptoms.

Both of the above are level 5 charts, written differently, but both acceptable. They each have 3 of 3 elements (Problem/Data/Risk) at level 5 — though remember, you only need 2 of 3 to bill at that level.